

ENGLISH FOR WORK / SEPTEMBER 2026

Nursing and Allied Health English



Teacher's guide

Teach practical workplace communication with specific cases, clear language targets, and a repeatable feedback process.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For nurses, charge nurses, physical therapists, occupational therapists, respiratory therapists, imaging staff, laboratory staff, care-team coordinators, and allied-health supervisors.

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

Your course at a glance

Use the lesson numbers to match this document with the website and the other guides.

01 Shift Handoffs and Clinical Prioritization

Give a handoff that the receiving person can confirm.

02 Patient Assessment and Escalation

Make a request with a clear action, purpose, and realistic timing.

03 Medication Safety and Allergy Clarification

Clarify missing information before responding.

04 Patient Education and Teach-Back

Explain a useful distinction in plain English.

05 Interprofessional Rounds

Include relevant voices and clarify the decision process.

06 Documentation and Charting

Separate observations, assumptions, and conclusions.

07 Difficult Families and Boundaries

Acknowledge a communication problem and give a corrected message.

08 Safety Events and Just Culture

Separate observations, assumptions, and conclusions.

Choose your pace

Quick practice: spend 15 minutes reading one conversation and rehearsing one scenario. Full lesson: allow 45-60 minutes for language work, three conversations, two speaking scenarios, and feedback.

Level support

B1 learners can use the frames and a partner. B2 learners can try a response before reading the model. C1 learners can add the harder follow-up and reduce preparation time. These are teaching suggestions, not certified CEFR level ratings.

After practicing: open the AI prompt workshop to choose a vocabulary, grammar, role-play, or new-dialogue activity.

A 60-minute lesson that works

Prepare the case and the learner pages. Keep the model response hidden until learners have attempted their own. Choose two field terms that are important to understanding the case.

0-5 minutes | Activate experience

Ask learners to name a comparable situation without sharing confidential details. Introduce the communication goal.

5-12 minutes | Read and sort facts

Read the case. Learners identify confirmed facts, unknowns, the audience, and the immediate communication need.

12-22 minutes | Notice and practice language

Explain the workshop pattern. Work through the vocabulary and editing example, then discuss both language checks and the reasons behind the choices.

22-35 minutes | 05 • Conversations

Read one ten-turn conversation aloud, notice a useful expression, then compare the other two. Identify each speaker's purpose and how the exchange ends.

35-47 minutes | 06 • Say it

Rehearse both speaking scenarios from the workbook. Give two minutes of preparation, switch roles, and use the success checks. Add the complication only after a successful first round.

47-55 minutes | Compare and revise

Reveal the model. Discuss alternative acceptable wording. Give one meaning correction and one language correction, then repeat.

55-60 minutes | Retrieve and transfer

Close the materials. Learners recall two expressions and identify one communication habit to use in another situation.

Adapt the session

For 45 minutes, study one complete conversation and rehearse one scenario; assign the others for oral follow-up. For 90 minutes, rehearse all three conversations and give both scenarios a second round. More time should produce more meaningful practice, not longer explanations.

LESSON 01 / FACILITATOR NOTES

Shift Handoffs and Clinical Prioritization

Give a handoff that the receiving person can confirm.

CASE

In a simulated handoff, the outgoing clinician has observations and background notes, but a requested result is still pending. The receiving clinician needs a clear transfer of information.

Teach this move

State the current situation, relevant background, open task, and receiving role. Ask for acknowledgment or repeat-back of the critical item. Sending a message and transferring responsibility are not always the same thing.

Vocabulary to check: SBAR, acuity, pending lab, watcher.

Model response

The result is still pending. I'll separate the current situation, relevant background, my assessment, and the request in this handoff. Please repeat back the outstanding task so we can confirm understanding.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revised sentence asks for an identifiable receiving team instead of leaving responsibility with an unspecified 'someone'.

Second-round challenge

The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

Language-check answers

1. B - Please confirm who will complete the outstanding check. It asks for an identifiable person or team responsible for the task.
2. A - So the result is pending and your team will follow up; is that correct? This repeats the status and responsibility and invites correction.

05 • Conversations

1. An SBAR handoff with a pending result - Fictional clinical simulation: case S4 reported tiredness at 14:00 and answered questions while seated. A blood count requested at 13:00 has no result displayed; sample receipt is unknown. Diagnosis and treatment remain outside this language exercise.
2. Clarifying the watcher label at shift change - Separate fictional handoff exercise: the training board labels case W3 a watcher, but the reason and current review status are not included.
3. A mobility handoff with an assessment still pending - Separate fictional allied-health simulation: a physiotherapy assessment has been requested but not completed; no mobility instructions are supplied.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 • Say it: second scenario | An owner missing from a pending review

A simulated handoff lists an occupational-therapy review as pending. The outgoing clinician knows it was requested but has not confirmed who accepted it. Practice transferring the information while resolving ownership; no clinical findings are supplied.

Role A: Outgoing nurse: identify the pending review and missing owner.

Role B: Incoming occupational therapist: ask for the request reference and agree how acceptance will be confirmed.

Possible opening: The review was requested, but I cannot yet confirm which clinician accepted it.

Success checks

- Distinguish a request from accepted responsibility.
- Do not invent findings or a clinical plan.
- Read back the agreed ownership check and follow-up route.

Add a complication: The next shift assumes pending means someone is already following it up.

LESSON 02 / FACILITATOR NOTES

Patient Assessment and Escalation

Make a request with a clear action, purpose, and realistic timing.

CASE

During a simulation, a patient reports feeling worse and the learner identifies a change from the earlier observation. The local escalation process is available.

Teach this move

State the action and the information needed. Explain why it matters and specify a deadline only when one is given or agreed. 'Could you' is polite; repeated apologies can obscure a legitimate request. Urgent situations may require a direct request.

Vocabulary to check: vital signs, rapid response, clinical deterioration, escalation.

Model response

I'm concerned because this is a change from the earlier observation. I need an assessment through our escalation process now. Here is what I observed and when it changed.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revision names the document and requested response. It avoids expressions that may be unfamiliar or ambiguous for an international audience.

Second-round challenge

The other person cannot meet the requested timing. Clarify an alternative without inventing authority to change a formal deadline.

Language-check answers

1. A - Send it no later than Thursday. 'By' sets the latest point, while 'after' refers to a later time.
2. C - Could you confirm the document version needed for the review? The requested action and information are explicit.

05 • Conversations

1. Requesting review for a reported change - Fictional clinical simulation: at 10:00 case C2 spoke in full sentences; at 10:20 the patient said I feel worse and paused repeatedly while answering. No vital-sign values, diagnosis, or treatment instruction is supplied; the local escalation process is available.

2. A callback without a confirmed receiver - Separate fictional escalation rehearsal: a nurse left a message about a change, but no clinician has acknowledged receipt; local escalation instructions are available.

3. An unexpected monitor value needs a clear review request - Separate fictional measurement simulation: the training monitor displayed a pulse of 78 beats per minute at 09:00 and 108 at 09:20. Measurement reliability and clinical significance are unassessed; these numbers are not action thresholds or treatment guidance.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | A concern interrupted during a call

In a clinical simulation, a learner starts reporting a change from baseline, but the call is interrupted before the request is acknowledged. No review has been confirmed. Practice reopening the call and stating the action needed using the local escalation route.

Role A: Bedside clinician: restate the observed concern and explicit review request.

Role B: Receiving charge nurse: confirm what you heard and clarify responsibility.

Possible opening: Our call ended before I confirmed that my request for review had been received.

Success checks

- State that review is requested, not merely that information exists.
- Use only supplied observations and avoid diagnosing the cause.
- Obtain acknowledgment and follow the local process for unresolved concerns.

Add a complication: The receiver initially thinks you are calling only with a routine status update.

LESSON 03 / FACILITATOR NOTES

Medication Safety and Allergy Clarification

Clarify missing information before responding.

CASE

A fictional medication record and the patient's account contain different allergy information. The learner must communicate the discrepancy through the clinical process.

Teach this move

Name the exact word, fact, or requirement you need clarified. Ask one focused question at a time. Repeat your understanding and give the other person a chance to correct it.

Vocabulary to check: allergy, contraindication, MAR, closed-loop communication.

Model response

The recorded allergy information differs from what the patient has told me. Could we verify the details through the appropriate clinical process? I will state the discrepancy clearly rather than assume either account is complete.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. After an introductory phrase such as 'Could you explain', the embedded question uses statement word order: "what this means".

Second-round challenge

The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

Language-check answers

1. C - Could you confirm when the review starts? The embedded question uses subject then verb: "the review starts".
2. B - Has the report been approved, or is it ready for review? This question distinguishes two meanings of 'ready' that affect the next action.

05 • Conversations

1. An allergy account that differs from the record - Fictional medication-reconciliation simulation: the chart says no known drug allergies. The patient reports a rash after penicillin years ago but cannot recall the date; prior verification and causality are unknown. No medication or treatment decision is supplied.

2. Unknown is not the same as no known allergies - Separate fictional chart review: an imported allergy field is blank; no patient interview or reconciliation has been completed.

3. A discussion mistaken for a medication instruction - Separate fictional medication-communication rehearsal: a colleague discussed a possible contraindication, but no reviewed instruction has been issued.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | An unreadable allergy entry

In a fictional charting simulation, an allergy entry contains an unreadable abbreviation and has not been reconciled. No substance or reaction is confirmed. Practice requesting clarification without guessing the abbreviation or making a medication decision.

Role A: Registered nurse: identify the exact unreadable entry and request source verification.

Role B: Clinical pharmacist: clarify what remains unknown and arrange the appropriate review.

Possible opening: I cannot reliably interpret this allergy abbreviation, so I need the source entry clarified.

Success checks

- Do not substitute a guessed substance or reaction.
- Distinguish the unclear entry from a confirmed allergy finding.
- Confirm the review route and use a read-back when verified information arrives.

Add a complication: A colleague offers a likely expansion from memory and asks you to use it.

LESSON 04 / FACILITATOR NOTES

Patient Education and Teach-Back

Explain a useful distinction in plain English.

CASE

A patient in a role-play nods during an explanation of an approved discharge sheet. The learner needs to check whether the explanation was clear.

Teach this move

Start with the reader's question. Explain one unfamiliar term with familiar words, then give a relevant example or contrast. Check understanding without asking only 'Do you understand?'.

Vocabulary to check: teach-back, discharge instructions, health literacy, adherence.

Model response

I want to check that I explained this clearly. In your own words, how will you follow the instructions when you get home? We can go over any part that is unclear.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revised sentence names the action in familiar words. Specialist vocabulary is useful when it adds precision, but repeating abstract nouns can hide the meaning.

Second-round challenge

Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Language-check answers

1. B - The backlog is the work still waiting to be completed. This gives the meaning in familiar words.
2. A - How would you describe the next step in your own words? An open request lets the listener show their understanding.

05 • Conversations

1. Teach-back after a nod - Fictional education rehearsal: the approved administrative sheet directs appointment-change requests to the Scheduling Office, not the ward desk. A standardized patient has nodded during the explanation. No clinical advice is practiced.
2. Repeating the words versus explaining the next step - Separate fictional teach-back simulation: an approved administrative sheet says to collect an attendance certificate from Reception after check-in, not before. The

standardized patient can repeat the heading but has not explained the sequence.

3. An unfamiliar word on the discharge sheet - Separate fictional allied-health education rehearsal: the approved administrative sheet lists a follow-up appointment for Tuesday at 09:00, with check-in at Reception. The standardized patient does not understand follow-up; no clinical instruction is practiced.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | A translation request before teach-back

In a fictional education exercise, the standardized patient says the approved discharge sheet is in a language they cannot read comfortably. An appropriate language-support request route is available, but support has not yet been arranged.

Role A: Discharge nurse: acknowledge the language need and request appropriate support without improvising clinical translation.

Role B: Interpreter coordinator: clarify the requested language and explain that availability still needs confirmation.

Possible opening: Before checking understanding, I need to arrange appropriate language support for this explanation.

Success checks

- Distinguish language access from demonstrated understanding.
- Do not invent interpreter availability or translate unsupplied clinical content.
- Agree the support request and a later teach-back check using approved information.

Add a complication: A colleague suggests treating the patient's polite nod as sufficient understanding.

LESSON 05 / FACILITATOR NOTES

Interprofessional Rounds

Include relevant voices and clarify the decision process.

CASE

In simulated rounds, one team member focuses on discharge timing and another raises an unresolved mobility concern.

Teach this move

Summarize the issue neutrally, invite missing perspectives, and distinguish discussion from decision. State who can decide and record any unresolved question. Do not assume silence means agreement.

Vocabulary to check: plan of care, functional status, case management, safe discharge.

Model response

Before we agree on timing, could we hear the mobility concern and the current assessment? Let's confirm the outstanding questions and who will address them in the care plan.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revised question gives people a way to raise a concern. Silence can have several meanings, especially in unfamiliar or unequal-power situations.

Second-round challenge

A participant interrupts or the meeting is running out of time. Protect a turn to speak and close with an accurate decision record.

Language-check answers

1. A - We haven't heard from the remote team yet; what is your view? It invites participation without interpreting silence as consent.
2. C - Approval is pending; the sponsor will review the two options. This distinguishes discussion from approval and identifies the next reviewer.

05 • Conversations

1. Making room for the mobility concern - Fictional interprofessional rounds: discharge timing is being discussed, while a mobility concern remains unresolved.

2. Equipment requested but not confirmed - Separate fictional rounds exercise: an occupational-therapy equipment request is pending, while case management is preparing discharge logistics.

3. Different meanings of ready during rounds - Separate fictional rounds rehearsal: nursing says education is ready to deliver, while the meeting chair interprets ready as all discharge work completed.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | A missing discipline at the huddle

In a simulated discharge huddle, the speech and language therapist is absent and their review is still pending. The group wants to finalize coordination arrangements. No assessment outcome is supplied. Facilitate a bounded discussion of the missing input.

Role A: Rounds facilitator: invite available perspectives and identify the missing review.

Role B: Case manager: explain logistical needs without substituting for the absent clinician.

Possible opening: We can discuss coordination needs, but the pending review must remain visible before an authorized decision.

Success checks

- Reject silence as evidence of clinical agreement.
- Separate logistics from the missing clinical assessment.
- Assign a contact owner and clarify who can confirm the plan.

Add a complication: A participant says the absent therapist's silence means agreement.

LESSON 06 / FACILITATOR NOTES

Documentation and Charting

Separate observations, assumptions, and conclusions.

CASE

A draft simulated note says a patient was "difficult." It contains no description of what was said or observed.

Teach this move

Say what the evidence shows, identify what is still unknown, and limit the conclusion to that evidence. Words such as 'may', 'suggests', and 'in this sample' are useful when the uncertainty is real. Do not soften an urgent, verified safety concern.

Vocabulary to check: charting, objective finding, intervention, patient response.

Model response

Let's replace that label with observable information. Record what the patient said or did, the relevant context, the action taken, and the response, following the documentation process.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revision states the finding and its scope. It avoids turning limited evidence into a universal claim.

Second-round challenge

Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Language-check answers

1. C - The change may have contributed to the delay. 'May have contributed' presents a possibility without claiming the cause is established.
2. B - In this pilot, the observed result was ... This locates the finding in the setting actually studied.

05 • Conversations

1. Replace difficult with an observed account - Fictional charting exercise: a draft note says difficult but includes no observed behavior or patient quotation.
2. A request for more explanation is not refusal - Separate fictional documentation simulation: the patient said, 'Please explain that again before we continue'; no further response is supplied.

3. Observed later does not mean caused by the intervention - Separate fictional note review: an intervention is documented and a later patient response is recorded, but the relationship has not been assessed.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | A colleague's observation in your note

In a fictional charting simulation, another clinician tells you a patient declined a conversation. You did not witness it, and the exact words are unknown. Discuss how to communicate the source and uncertainty before any documentation is finalized.

Role A: Receiving clinician: distinguish secondhand information from your own observation.

Role B: Reporting clinician: clarify what you actually observed without adding motives.

Possible opening: I did not witness the conversation, so I need to distinguish your report from my own findings.

Success checks

- Attribute the report to its actual source.
- Avoid inventing quotations, emotion, or motivation.
- Agree which observed details need clarification through the documentation process.

Add a complication: A colleague suggests writing the patient was angry, although no behavior supporting that description is supplied.

LESSON 07 / FACILITATOR NOTES

Difficult Families and Boundaries

Acknowledge a communication problem and give a corrected message.

CASE

A family member in a role-play says, "Nobody tells us anything." The learner can explain their role and arrange an appropriate conversation.

Teach this move

Name what was unclear or wrong, acknowledge its effect, and correct it. A brief apology can help, followed by a practical next step. Avoid explaining away the other person's reaction or promising an outcome you cannot control.

Vocabulary to check: scope of practice, privacy, family meeting, boundary.

Model response

I can hear that the lack of information is frustrating. Let me explain what I can clarify and arrange contact with the appropriate team member for the questions outside my role.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revision takes responsibility for the message without blaming the listener. It identifies what needs to be corrected.

Second-round challenge

The listener remains disappointed after the correction. Acknowledge the impact and restate the available next step calmly.

Language-check answers

1. B - I'm sorry my earlier wording was unclear. This acknowledges the speaker's contribution to the problem.
2. A - The corrected date and the current confirmation status. A repair needs usable corrected information, including any uncertainty.

05 • Conversations

1. Responding to nobody tells us anything - Fictional family-communication rehearsal between a nurse and a patient liaison: a family member has said nobody tells us anything; sharing permissions are unconfirmed.
2. Correcting an unconfirmed family-meeting time - Separate fictional communication case: a nurse said the family meeting would be at two, but clinician availability has not been confirmed.

3. An apology without crossing a privacy boundary - Separate fictional liaison call: an earlier message implied any relative could receive updates, but the appropriate sharing permissions have not been verified.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | A question redirected without explanation

In a fictional family liaison exercise, a relative was transferred between two departments without being told why. Their clinical question remains unanswered, and the responsible clinician has not been identified. Practice a repair conversation between staff.

Role A: Patient liaison: acknowledge the confusing transfers and ask for a named contact route.

Role B: Ward coordinator: clarify your own role and arrange identification of the appropriate clinician.

Possible opening: We transferred the question without explaining the route, so we need to give the family a clearer next step.

Success checks

- Acknowledge the communication failure without blaming the relative.
- Keep the response within role and privacy boundaries.
- Agree who identifies and confirms the appropriate contact.

Add a complication: A colleague proposes answering the clinical question from memory to end the complaint.

LESSON 08 / FACILITATOR NOTES

Safety Events and Just Culture

Separate observations, assumptions, and conclusions.

CASE

A simulated safety report includes a fall and an assumption that a colleague was careless. The review has not established the contributing factors.

Teach this move

Say what the evidence shows, identify what is still unknown, and limit the conclusion to that evidence. Words such as 'may', 'suggests', and 'in this sample' are useful when the uncertainty is real. Do not soften an urgent, verified safety concern.

Vocabulary to check: fall risk, incident report, just culture, root cause.

Model response

The report should describe the event and available observations. We have not established the contributing factors. I'll remove the unsupported judgment and use the reporting process to raise the concern.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revision states the finding and its scope. It avoids turning limited evidence into a universal claim.

Second-round challenge

Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Language-check answers

1. A - The change may have contributed to the delay. 'May have contributed' presents a possibility without claiming the cause is established.
2. C - In this pilot, the observed result was ... This locates the finding in the setting actually studied.

05 · Conversations

1. A fall report without a blame conclusion - Fictional safety-report review: a fall is reported, but contributing factors are unestablished; a draft calls a colleague careless.
2. A missing entry after an intercepted mix-up - Separate fictional safety simulation: a label mix-up was intercepted before use; the check's documentation is missing, and the sequence is not fully established.

3. One refresher session does not prove prevention - Separate fictional safety review: staff attended a refresher after an incident; no effectiveness assessment or root-cause finding has been completed.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | Two accounts of the same safety event

In a fictional incident review, two clinicians remember a call occurring at different times. Neither timestamp has been verified, and no causal conclusion exists. Discuss how to preserve both accounts and seek corroboration.

Role A: Safety interviewer: describe the disagreement neutrally and request available records.

Role B: Clinical supervisor: separate recollection from verified timing and keep the review open.

Possible opening: The accounts differ on the call time, so we need to preserve both while checking the records.

Success checks

- Do not treat seniority as proof that an account is correct.
- Distinguish recalled timing from verified evidence.
- Agree a corroboration step without assigning blame or a root cause.

Add a complication: A participant says the newer clinician's account should be discarded solely because they have less experience.

Give feedback that helps

Score each criterion 0, 1, or 2. This is a practice rubric, not a professional qualification or CEFR test. Do not award or remove points for a particular accent.

Meaning and accuracy

Keeps the case facts accurate; clearly separates confirmed and unknown information.

Clarity and language

Uses understandable sentences and explains specialist terms when the listener needs it.

Interaction and tone

Responds to the other person's question, checks understanding, and uses an appropriate tone.

Task completion

Makes the requested action or unresolved question clear without inventing authority or facts.

Use the same scale for each criterion

0 - Not yet: The reader or listener cannot yet recover the intended meaning or action.

1 - With support: The message works after a prompt, clarification, or revision.

2 - Independently: The message is clear and accurate without a prompt.

Feedback sequence

First, identify a successful phrase. Next, identify one point where meaning or accuracy needs work. Offer one useful language correction, allow a repeat, and compare the two attempts. A total out of eight describes this performance only; do not translate it into a proficiency level.

Final challenge

Choose a case not rehearsed today. The learner gives a one-minute response, answers two follow-up questions, and writes a 70-110 word message. Add the lesson's second-round challenge. Compare the result with an earlier attempt using the four criteria.

Use the language responsibly

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

AHRQ: SBAR communication tool

<https://www.ahrq.gov/teamstepps-program/curriculum/communication/tools/sbar.html>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.